Guidance for the diagnosis of AGW

In terms of diagnosis, the key challenge is ensuring that anogenital warts (AGW) are correctly identified. AGW are typically diagnosed clinically based on their characteristic morphology and anogenital location. In most cases, diagnostic testing is not required when the lesions are typical in appearance.

Clinical Evaluation

A thorough history and physical examination are essential for accurate diagnosis and effective management. The following elements should be included during the initial patient consultation:

Patient History Checklist:

- **Symptoms**: Presence of itching, bleeding, discomfort, or pain
- **Duration**: Onset and progression of lesions
- **Number and location** of warts (e.g., penile, scrotal, vulvar, vaginal, cervical, perianal, anal)
- **Morphology**: Appearance (e.g., exophytic, papular, keratotic, flat)
- **Previous episodes** of AGW and history of recurrence
- **Prior treatments** used (type, duration, response, side effects)
- **Sexual history**: Number of partners, condom use, recent new partners
- **HPV vaccination status**
- **Immunocompromising conditions** (e.g., HIV infection, immunosuppressive therapy)
- **Psychosocial impact**: Emotional distress, anxiety, shame, effect on relationships or sexual activity

Physical Examination:

- Inspection of external genitalia, perianal area, and adjacent skin under adequate lighting
- Use of a magnifying lens if necessary
- Application of 3–5% acetic acid (optional) to highlight subclinical lesions (though this is not routinely recommended due to low specificity)
- For women: speculum examination to assess vaginal and cervical involvement
- For men who have sex with men (MSM) or individuals with anal receptive intercourse: digital rectal examination and/or anoscopy to evaluate for internal anal warts

Differential Diagnosis

Conditions that may mimic AGW include: - **Pearly penile papules** - **Hirsuties papillaris genitalis** - **Sebaceous hyperplasia** - **Molluscum contagiosum** - **Condyloma lata** (syphilitic lesion – requires serological testing) - **Squamous cell carcinoma** (especially in immunocompromised patients or long-standing lesions) - **Vulvar intraepithelial neoplasia (VIN)** or **anal intraepithelial neoplasia (AIN)**

Biopsy should be considered in cases of: - Atypical appearance (e.g., pigmentation, ulceration, induration) - Poor response to standard therapy - Suspected malignancy - Immunocompromised patients - Perianal warts in children (to rule out sexual abuse)

Role of Diagnostic Testing

- **HPV DNA testing** is not recommended for routine diagnosis of AGW, as it does not change management.
- **Acetic acid testing** has limited utility and may lead to false positives; its use is discouraged outside of specific settings such as colposcopy.
- **Serological tests** for syphilis should be performed if condyloma lata is suspected.

- **HIV testing** should be offered, particularly in patients with extensive, recurrent, or atypical warts.

Patient Communication: Frequently Asked Questions (FAQs)

To support patient understanding and reduce anxiety, physicians should address common concerns:

- **"Are anogenital warts caused by cancer-causing viruses?"**
  No, most AGW are caused by low-risk HPV types 6 and 11, which do not cause cancer.
- **"Can I pass this to my partner?"**
  Yes, HPV can be transmitted through skin-to-skin contact, even without visible warts.
- **"Will I always have this?"**
  Most people clear the infection within months to a couple of years. Recurrences are common but tend to decrease over time.
- **"Should my partner be examined?"**
  Routine screening of asymptomatic partners is not recommended, but they should be informed so they can seek care if symptoms develop.
- **"Does this mean my partner was unfaithful?"**
  Not necessarily. HPV can remain dormant for months or years before causing symptoms.
- **"Can I still have sex?"**
  Yes, but using condoms may reduce transmission risk (though they do not fully protect due to uncovered skin areas).

This structured approach to diagnosis ensures accurate identification of AGW, appropriate exclusion of mimicking conditions, and effective patient counseling.